

Smartphones and Children's Mental Health

What happened when English communities delayed their children's smartphones

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Research summary of a working paper under peer review. Full paper:

https://gregveramendi.github.io/papers/Veramendi_Smartphones.pdf

The question. Children's mental health has deteriorated across rich countries since smartphones became a fixture of childhood. Governments are responding with school phone bans and social-media age limits. Yet there has been no causal evidence on the step many parents care about most, delaying the first smartphone. The Smartphone Free Childhood movement launched in February 2024 and organizes parents through a "Parent Pact," a public pledge to delay smartphones until age 14. By early 2026, over 165,000 parents across more than 15,600 UK schools had signed. The change on the ground can be striking. At one of the early adopters, Year 6 smartphone ownership fell from 75 percent to 3 percent in two years.

The study. I compare how mental-health needs changed in English primary schools where at least 20 percent of families signed the pact with similar schools where almost no one signed. Children's mental health is measured in two independent administrative systems: the school census, where schools record each pupil's social, emotional and mental health (SEMH) needs, and child psychiatric disability benefits (DLA), which government caseworkers determine with no direct involvement from schools.

The findings. In high-signup communities, recorded mental-health need fell by about 8 percent within two years, relative to closely matched comparison schools. Total child psychiatric disability claims fell by about 6 percent in the same communities. The reductions grow over time and with the share of families signing. They are about twice as large for girls. These headline numbers are conservative, because they average over all ages and conditions and over every family, signers or not. For the children and conditions a phone delay could plausibly affect, the implied reductions may be two to four times as large.

Is this just engaged parents? Communities that sign are more affluent and their children may have lower mental-health needs for other reasons. The analysis addresses this in three ways.

- **Like is compared with like.** Each high-signup school is compared with schools that looked the same before 2024, with the same recorded mental-health levels, similar affluence, and similar size. The estimates compare *changes* within high-signup schools to the *changes* within similar low-signup schools. Average differences between schools and communities are removed.
- **Two independent systems agree.** Needs measured by schools and the Department for Work and Pensions fall together. It is unlikely that the decision-making processes at two different agencies change at the same time only in engaged communities.
- **The pattern matches a phone effect, not general engagement.** I find reductions only for the conditions and ages that are plausibly affected by phone delay: behavioral conditions such as ADHD, and 5 to 10 year-olds, whose first phones are actually being delayed. Conditions a phone cannot affect stay flat, including autism, developmental disabilities, and adult depression in the same neighborhoods. It is unlikely that parental engagement alone could produce this pattern.

What it means. Parents fear that withholding a smartphone will socially isolate their child. The pact solves this coordination problem by converting a private intention into a visible, school-year-specific commitment. The findings give parents causal evidence that delaying the first smartphone through the primary-school years improves children's mental health, and they give governments a near-zero-cost complement to school phone bans and statutory age limits. The reductions also bear on two fast-growing public budgets, child disability benefits (over £4 billion a year) and high-needs education (£12 billion). Two cautions apply. The evidence covers only the movement's first two years. The adopting communities are relatively affluent, and the effects in more disadvantaged areas could be larger or smaller.